



What \$373 Has To Buy

Alberta sets the accommodation charge in continuing care, and sets the disposable income left afterwards. This is what that money still has to cover.

If you move into a Type A or Type B continuing care home in Alberta, your care is publicly funded. What you pay is the accommodation charge, and the province sets the maximum.

If you receive AISH, ADAP or Income Support, the province also sets what you keep. **As of January 2026, residents eligible for these benefits are assured of at least \$373 per month in disposable income after the accommodation charge is paid.**

Both of those numbers are published. What is not published anywhere, in one place, is what the \$373 still has to cover.

The two numbers, as Alberta listed them

As of **1 April 2026**, Alberta listed maximum daily accommodation charges of **\$70.60 shared room**, \$81.60 private, \$97.70 one-bedroom suite, \$112.45 multi-bedroom suite. A shared room works out at roughly **\$2,148 a month**.

Those rates **rose again on 1 August 2026** by an inflation adjustment of 1.8 per cent. Read the current table off alberta.ca/continuing-care-accommodation-charges. **The charge is adjusted every year.**

AISH is \$1,940 a month. ADAP is \$1,740. The cheapest accommodation charge in continuing care is more than either benefit. A government subsidy carries the difference, and the resident is left at the floor.

The floor is \$373. Nothing located establishes that the floor rises with the charge. The charge is adjusted annually. Whether the floor is adjusted is an open question and we are asking it in writing.

What the accommodation charge covers

Alberta describes accommodation charges as covering the cost of providing services like **rooms, meals, laundry, housekeeping, utilities, building maintenance and general administration.**

That is the list. It is shorter than most people assume when they hear that a facility costs over two thousand dollars a month.

What it does not cover

Everything below comes out of the \$373, or out of a family member's pocket, or it does not happen.

What it is	Where it sits	Who pays
A telephone line in your room	Not in the published list of what the accommodation charge covers.	Where a resident cannot pay for a line, the phone available is generally the one at the desk.
Television and cable	Not in the published list.	Paid by the resident, or not had.
Clothing and replacing it	Not in the published list. Laundry is covered. Clothes are not.	Out of the \$373.
Incontinence supplies	Not in the published list, and not a health service under the standards.	A monthly supply can take a large share of \$373.

What it is	Where it sits	Who pays
Personal toiletries	Beyond what the home supplies.	Out of the \$373.
Haircuts, foot care, and similar	Not in the published list.	Out of the \$373.
A bed you can safely use	The standards require the operator to provide assessed assistive equipment or refer you to a service that can provide it.	A referral discharges the obligation. The equipment program then has its own rules, its own cost share and its own wait.
Getting anywhere	Not in the published list.	Out of the \$373.

The equipment clause is the one to read twice

Section 6.1 of the Continuing Care Health Service Standards says an Operator must ensure a Client is **provided** with assistive equipment they have been assessed as requiring, **or referred** to a service which can provide it.

The standards list assistive equipment as including walkers, wheelchairs, canes, scooters, motorised wheelchairs, grab bars, bath chairs and mechanical lifts.

That single word "or" means a referral satisfies the standard. A resident can be lawfully accommodated, with the operator fully compliant, while the equipment they were assessed as needing sits in another program's queue.

What to do: ask for the assessment in writing, ask whether the operator is providing or referring, and ask for the referral date. If it is a referral, you are now dealing with two systems, and only one of them has a standard attached.

The part that makes the rest unenforceable

Section 19 of the health service standards requires an operator to give a resident, or their legal representative or family, **written information about outside complaint and concerns resolution processes** — not only the operator's own process — when care begins and any time it is asked for.

That is a real entitlement and it should be used. But look at what those outside routes are.

- **AHS Patient Relations.** A telephone service.
- **The Office of the Alberta Health Advocates.** A telephone service.
- **The Alberta Ombudsman.** A telephone service.
- **Protection for Persons in Care.** A telephone reporting line.
- **The Appeals Secretariat.** A telephone service.

A telephone line in a resident's room is not covered by the accommodation charge. It comes out of \$373 a month, alongside clothing, incontinence supplies and everything else on the previous page.

Said plainly

The province guarantees the right to complain, and prices the means of complaining outside the income it guarantees.

Where a resident cannot afford a line of their own, the telephone within reach is generally the one at the desk — among the people a complaint would be about.

This is not a gap in the standards. The standards are met. It is the charge structure and the income floor producing an isolation that makes the standards difficult to enforce.

All three pieces are published by the Government of Alberta: what the accommodation charge covers, the \$373 floor, and the complaint routes listed as telephone services. They have simply never appeared on the same page.

Three absences in the standards, named as absences

These are not oversights we are alleging. They are things that are not in the document, checked as of 13 September 2026.

1. There is no minimum staffing level

The standards cover criminal record checks, competency profiles, supervision and training in detail. Section 18 requires contingency plans to contain measures ensuring sufficient staff **during a disruption**. For normal operations, no published minimum ratio was located. **"Sufficient staff" is not defined or measured.**

2. There is no property or security standard

Nothing located in the standards requires a resident's belongings or money to be secured, requires a lockable door or locker in a shared room, or makes the loss of a resident's money a reportable incident. Section 20.5 lists reportable incidents; theft of a resident's money is not among them.

Section 4.4 requires an operator to provide information on trusteeship and enduring powers of attorney **where the client is assessed as needing it**. There is no requirement that anyone hold or safeguard the money itself.

3. Every safeguard assumes somebody is there

Read the protections in the standards and notice who they run to. Information before a restraint is used, information about a secure space, participation in the interdisciplinary team, access to the care plan, the concerns process — each is owed to **the Client or the Client's legal representative**.

For a resident with no family, no friend and no appointed representative, the chain has nobody on the end of it. **The regulation assumes a person on the outside who is checking.** For a great many people in continuing care, there is not one.

If you are supporting someone in a home

Six things, all grounded in the standards, all askable in writing.

- **Ask for the written statement of funded and unfunded services and any costs assigned** to the resident. Section 4.1 requires it when care begins, in writing. It should list precisely what the accommodation charge does and does not cover at that home.
- **Ask whether the financial screening has been done** for the accommodation charge subsidy. It is not automatic if nobody assesses for it.
- **Ask for the equipment assessment in writing**, and whether the operator is providing or referring. Get the referral date.
- **Ask for the written information about outside complaint routes**. Section 19.2 requires it on admission and on request. Having it on paper matters when the phone is at the desk.
- **Ask to be recorded as the resident's representative on the interdisciplinary team**, and to be invited to the care plan review. The standards allow the client to invite individuals of their choosing.
- **Ask how the resident's money is held**, who has access to it, and what record is kept. There is no standard requiring an answer. Ask anyway, in writing, and keep the reply.

And before any of this becomes necessary

Most people arrive in continuing care because nobody told them, years earlier, what else existed.

The assessment that opens a continuing care home is the same assessment that opens home care. One call to Health Link at 811. No referral, no cost, no form, and it has nothing to do with your age.

Home modifications are funded. Up to \$12,000 a year and \$24,000 over ten years through the Residential Access Modification Program, as of 21 August 2026. Renters qualify with a landlord-signed agreement. It can also fund modifications to a caregiver's home.

Assessed home care hours carry no client fee in Alberta.

Ring before the crisis, not during it. The bag of clothes is what happens when the first conversation about this is also the urgent one.

What we could not confirm, said plainly

Whether the \$373 floor is adjusted annually. The accommodation charge is. Nothing located says the floor is. This is a written question outstanding with the ministry.

The August 2026 accommodation charges are not printed here. The 1.8 per cent increase is confirmed. The current table should be read off alberta.ca rather than calculated.

What the province pays operators per resident. Alberta publishes a maximum full accommodation revenue figure that includes the resident charge plus a government subsidy. That figure has not been read and is not stated here.

Practice at individual homes is not covered by this document. What is included in an accommodation charge can differ between operators. That is exactly why the written statement under section 4.1 matters.

Everything here is drawn from the Continuing Care Act and its Health Service Standards, and from Alberta.ca pages, as they stood on 13 September 2026.

*A person is not a resident. The things that make someone themselves are not an extra.
Ask what the charge covers. Ask in writing. Keep the answer.*

The Alberta Disability System Breakdown

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Compiled September 2026 from the Continuing Care Health Service Standards (Alberta Health, April 2024) and Alberta.ca continuing care and accommodation charge pages. Verify anything time-sensitive before relying on it.